SOURCE SECTION 1

Simulated Teaching Hospital

Medical Record Copy Format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record number: SIM-ASC-20260901

Inpatient Medical Record Copy · Expert-Review Edition

Case Theme

Worsening ascites in decompensated hepatitis B cirrhosis, complicated by spontaneous bacterial peritonitis and acute kidney injury; discharged with improvement after anti-infective therapy, volume management, and treatment of ascites.

Patient and Record Information

Field	Value	Field	Value
Name / Sex / Age	Wang Jianhua (fictional) / Male / 56 years	Department / Bed number	Gastroenterology (Hepatology Ward) / Simulated bed 12
Medical record number	SIM-ASC-20260901	Admission / Discharge	2026-09-01 09:20 / 2026-09-09 10:00
Length of stay	8 days (spanning 9 calendar dates)	Document version	v1.1 / 2026-09-15 / Pending clinical expert review
Purpose	Clinical reasoning training for senior residents and clinical expert review		

Nature of the Document

All identities, clinical conditions, laboratory values, microbiological results, and procedure details in this booklet are synthetic teaching material and do not come from a real patient. This booklet is a complete answer-key medical record containing the final diagnoses and should not be distributed in full to trainees during the initial patient assessment stage of an examination.

Images and Signatures

The ultrasound images are AI-generated illustrations, and the electrocardiogram is an algorithmically drawn illustration. The written reports are part of the case scenario; the images cannot validate the measurements and cannot serve as a gold standard for an examination assessing interpretation of real clinical images. There are no real hospital logos or seals; the footer contains simulated handwritten signatures of fictional physicians.

Document Contents

Medical record front sheet, admission record, initial and daily progress notes, laboratory and investigation reports, abdominal paracentesis and therapeutic ascitic fluid drainage records, summary of medical order execution, nursing monitoring, and discharge record. Teaching objectives, staged disclosure materials, and review forms are provided in a separate booklet.

SIM-ASC-20260901 | Simulated teaching case · Not a real medical document

Simulation Teaching Hospital

Medical record copy format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record number: SIM-ASC-20260901

Inpatient Medical Record Front Sheet

Registration Details

Registration item	Recorded information
Name	Wang Jianhua (fictional)
Date of birth / Age	1970-04-18 / 56 years
Sex / Ethnicity / Marital status	Male / Han Chinese / Married
Occupation / Place of origin	Warehouse administrator / Chengdu, Sichuan
Current address	Apartment 502, Building 3, Jinhe Yuan, 36 Jinhe Road, Wuhou District, Chengdu, Sichuan Province
Contact / Source of history	Wife (family member role A) / Patient, supplemented by his wife; generally reliable
Route of admission	Admitted from the gastroenterology outpatient clinic
Admission diagnoses	Decompensated hepatitis B cirrhosis with worsening ascites; intra-abdominal infection to be ruled out; renal dysfunction, cause to be investigated
Principal discharge diagnosis	Decompensated hepatitis B cirrhosis with ascites
Other discharge diagnoses	Spontaneous bacterial peritonitis (Escherichia coli); stage 1 acute kidney injury (resolved); hyponatremia (improved); portal hypertension and splenomegaly with thrombocytopenia; hypoalbuminemia
Diagnostic and therapeutic procedures	09-01 diagnostic paracentesis; 09-03 repeat paracentesis for reassessment; 09-04 therapeutic paracentesis with removal of 6000 mL of ascitic fluid
Surgery / Transfusion	No surgery, TIPS, or liver transplantation performed; no red blood cells, plasma, or platelets transfused. See medication orders for human albumin administration.
Drug allergies / Outcome	Denies known drug allergies / Discharged with improvement

Diagnostic codes, costs, actual identification numbers, signatures, and seals have not been fabricated. This page is a teaching medical record front-sheet format, not a front sheet for administrative reporting.

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SOURCE SECTION 3

Simulated Teaching Hospital

Medical record copy format / v1.1 / 2026-09-15 / Pending clinical expert review

Patient and administrative information

Field	Value	Field	Value
Name	Wang Jianhua (fictional)	Sex	Male
Age	56 years	Department	Gastroenterology
Medical record number	SIM-ASC-20260901	Service	Gastroenterology
Recorded by	Zhao Heng	Reviewer	Shen Yan

Simulated handwritten-style signature of a fictional physician · For teaching use

Admission Record (I) · Chief Complaint and History of Present Illness

Recorded at: 2026-09-01 10:00

Chief Complaint

Recurrent abdominal distension for 6 months, worsening with lower-extremity edema for 10 days and decreased urine output for 2 days.

History of Present Illness

The patient was found to be positive for hepatitis B surface antigen during a physical examination 20 years ago. Hepatitis B–related cirrhosis was diagnosed 3 years ago based on liver imaging and blood tests. Abdominal distension and bilateral lower-extremity edema first developed 6 months ago and improved after outpatient antiviral therapy and oral diuretics. He was not hospitalized and did not undergo paracentesis. His abdominal girth had remained essentially stable over the past month. On August 20, his weight was 68 kg and creatinine was 78 μmol/L.

Ten days ago, after a group meal and eating a relatively large amount of preserved foods, his abdominal distension gradually worsened, with a tighter waistband and early satiety. Edema of both ankles extended to the lower legs. From August 28 to 30, he increased furosemide on his own from 20 mg/day to 40 mg/day, while spironolactone remained at 50 mg/day, without significant relief of abdominal distension. Over the past 3 days, his food intake has been approximately half his usual amount, and water intake approximately 1000 mL/day. Over the past 2 days, his estimated urine output has decreased from approximately 1500 mL/day to 800-1000 mL/day, with dark urine and increased fatigue after activity.

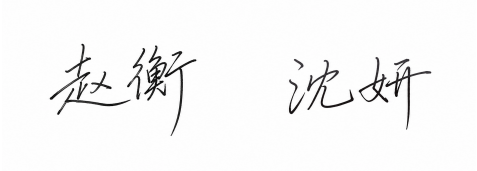
He reports no chills; his highest self-measured temperature was 37.4°C. He has only mild, diffuse, dull abdominal pain, without persistent, localized severe pain. There has been no vomiting, diarrhea, hematemesis, or melena. Recent stools have been yellowish-brown, once daily. There has been no cough, sputum production, or dysuria, and no sleep–wake reversal, abnormal behavior, or confusion. He can lie flat at night and has no chest pain or orthopnea. Marked ascites and relatively low blood pressure were noted at today's outpatient visit, prompting admission.

Recent Medications and Exposures

He regularly takes entecavir 1 mg/day (with prior decompensation), spironolactone 50 mg/day, and furosemide 20 mg/day. He had not yet taken his diuretics on the day of admission. He denies recent use of NSAIDs, Chinese herbal medicines, contrast agents, or other nephrotoxic drugs. He has not been hospitalized or used antibacterial drugs in the past 3 months and has no known history of colonization with resistant organisms.

Since symptom onset, he has felt somewhat less well, his sleep has been affected by abdominal distension, and his appetite has decreased. Recent weight gain is considered primarily attributable to fluid retention; nutritional risk cannot be excluded on the basis of weight gain.

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Simulated Teaching Hospital

Medical Record Copy Format / v1.1 / 2026-09-15 / Pending clinical expert review

Patient and record information

Field	Value	Field	Value
Name	Wang Jianhua (fictional)	Sex	Male
Age	56 years	Department	Gastroenterology
Medical record number	SIM-ASC-20260901	Service	Gastroenterology
Recorded by	Zhao Heng	Reviewer	Shen Yan

Simulated handwritten-style signatures of fictional physicians · For teaching use

Admission Record (II) · Past Medical, Personal, and Family History

Liver Disease History

Approximately 20 years of hepatitis B; follow-up was irregular in the early years. After developing ascites 6 months ago, he started entecavir 1 mg once daily on an empty stomach. He denies missed doses or prior use of nucleoside agents such as lamivudine. No history of spontaneous bacterial peritonitis (SBP), hepatic encephalopathy, gastrointestinal bleeding, liver cancer, or portal vein thrombosis.

Previous Investigations

Summary of outpatient gastroscopy on 2026-06-15: small varices in the lower esophagus, with no red signs or active bleeding; mild portal hypertensive gastropathy. A nonselective beta-blocker has not yet been initiated because outpatient systolic blood pressure was usually 95–100 mmHg, with occasional dizziness; reassessment by a liver specialist is planned. On 2026-08-20, creatinine was 78 μmol/L, sodium 135 mmol/L, and potassium 4.2 mmol/L; at that time, there was no evident edema, and weight was 68 kg. The above is a summary of historical records; no original gastroscopy images were separately fabricated.

Other Past Medical History

He denies a history of hypertension, diabetes mellitus, coronary artery disease, heart failure, chronic kidney disease, tuberculosis, or malignancy. No history of surgery, trauma, or blood transfusion. Vaccination details are unknown. He denies known food or drug allergies.

Personal History

Born in and a long-term resident of Chengdu, Sichuan. Married, lives with his wife, and has one son. Previous smoking history of approximately 10 pack-years; quit 5 years ago. Previously drank small amounts of alcohol and stopped after the diagnosis of cirrhosis. No intravenous drug use, tattoos, or recent contact with contaminated water in an endemic area. His work mainly involves standing and light manual handling.

Family History

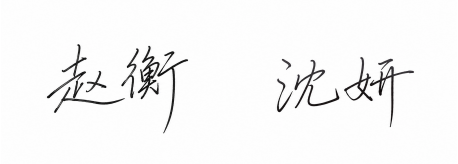
His mother is hepatitis B surface antigen-positive; no history of liver cancer is known in her. His father has hypertension. No known hereditary liver or kidney disease.

Review of Systems

No recent significant weight loss or night sweats; no chest tightness, palpitations, or unilateral lower-limb swelling and pain; no frequent small-volume urination, hematuria, or foamy urine; no rash or joint swelling and pain; no visual abnormalities or focal neurological symptoms.

Recording physician: Zhao Heng (simulated signature for teaching)

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Simulated Teaching Hospital

Medical Record Copy Format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record number: SIM-ASC-20260901

Gastroenterology · Recorded by: Zhao Heng Reviewer: Shen Yan

Simulated handwritten-style signatures of fictional physicians · For teaching use

Admission Record (III) · Physical Examination

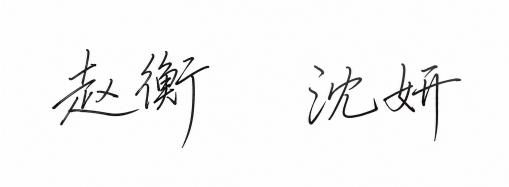
Physical examination findings

Item	Result
Vital signs	T 37.3°C; P 96 beats/min; R 20 breaths/min; BP 98/62 mmHg; SpO2 97% (room air)
Height / Weight / Abdominal circumference	170 cm / 76.0 kg / 106 cm; recent weight of 68 kg when free of edema used as estimated dry weight
General appearance	Alert, fully oriented, with chronic liver disease facies; mild scleral icterus, palmar erythema, and spider angiomas on the upper chest
Nutrition and skin	Reduced muscle bulk in the temporal regions and upper arms; skin intact, with no active ecchymoses or foci of infection
Head and neck	No jugular venous distention; no palpable superficial lymph node enlargement
Heart and lungs	Heart rate 96 beats/min, regular rhythm, with no obvious murmurs heard; clear breath sounds bilaterally, with no crackles
Abdominal inspection and palpation	Abdomen distended, abdominal wall slightly tense, umbilicus flat; mild diffuse tenderness, without rebound tenderness or guarding; liver not palpable below the costal margin; spleen palpable approximately 2 cm below the costal margin
Abdominal percussion and auscultation	Positive shifting dullness and fluid wave; bowel sounds 4/min; no obvious vascular bruits
Urinary system and extremities	No tenderness to percussion over the renal areas; symmetrical 2+ pitting edema of both lower legs; extremities warm
Nervous system	No asterixis, pathological reflexes, or focal signs

Initial Assessment on Admission

Manifestations of decompensated cirrhosis and marked ascites are present. Although there is no high fever, mild abdominal pain, decreased urine output, and relatively low blood pressure warrant prioritizing assessment for intra-abdominal infection and circulatory perfusion; this must not be managed solely as “hypoproteinemic edema.”

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SOURCE SECTION 6

Simulated Teaching Hospital

Medical record copy format / v1.1 / 2026-09-15 / Pending clinical expert review

Patient and record information

Field	Value	Field	Value
Name	Wang Jianhua (fictional)	Sex	Male
Age	56 years	Department	Gastroenterology
Medical record number	SIM-ASC-20260901	Service	Gastroenterology
Recorded by	Zhao Heng	Reviewer	Shen Yan

Simulated handwritten-style signature of a fictional physician · For teaching use

Initial Progress Note

Recorded at: 2026-09-01 10:20 (ascitic fluid results had not yet been reported at this time)

Case Features

A 56-year-old man with hepatitis B cirrhosis and a history of ascites presents with worsening abdominal distension and edema for 10 days and reduced urine output for 2 days. Recently, he has consumed a high-salt diet, increased his diuretic doses on his own, and reduced his food intake. Examination shows ascites, mild abdominal pain, and low-normal blood pressure, without signs of peritoneal irritation or impaired consciousness.

Preliminary Diagnoses and Supporting Evidence

1. Decompensated hepatitis B cirrhosis with worsening ascites: supported by the history of cirrhosis and ascites and the current physical findings. 2. Intra-abdominal infection to be ruled out: absence of fever does not exclude spontaneous bacterial peritonitis (SBP). 3. Renal dysfunction of undetermined cause: comparison with previous baseline renal function and urine output is required.

Differential Diagnosis

Causes of ascites include portal hypertensive, cardiac, malignant, and tuberculous ascites. First assess the serum–ascites albumin gradient (SAAG) and ascitic fluid protein, then select further investigations according to the findings. The differential diagnosis of oliguria includes hypoperfusion related to inadequate intake or diuretics, infection-related kidney injury, hepatorenal syndrome–acute kidney injury (HRS-AKI), renal parenchymal injury, and obstruction. Persistent localized abdominal pain or polymicrobial cultures should raise concern for secondary peritonitis.

Diagnostic and Treatment Plan

Withhold spironolactone and furosemide; record blood pressure, urine output, weight, and fluid intake and output. Urgently obtain a complete blood count, liver and renal function tests, electrolytes, coagulation studies, CRP, lactate, urinalysis/urine sediment examination, and blood cultures. Perform ultrasound assessment and diagnostic paracentesis as early as possible; send ascitic fluid for a differential cell count, albumin, total protein, glucose, LDH, and culture. Bedside inoculation of ascitic fluid must be completed before antimicrobial administration.

Assess for signs of peritoneal irritation and evaluate hemodynamic and respiratory status. Decide on antimicrobial treatment immediately after the rapid ascitic fluid results become available, without waiting for final culture results. Determine the albumin and fluid strategy according to clinical volume status; do not overlook circulatory perfusion because of the large volume of ascites, and do not indiscriminately administer large volumes of fluid. Continue antiviral therapy and review the dose in light of renal function. Explain the diagnosis and the purpose of paracentesis to the patient and family.

Senior review: Shen Yan; recorded by: Zhao Heng (both signatures simulated for teaching)

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SOURCE SECTION 7

Simulated Teaching Hospital

Medical Record Copy Format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) Sex: Male Age: 56 years Department: Gastroenterology Medical record number: SIM-ASC-20260901

Laboratory Department · Recorded by: Lin Yan Reviewer: Su He

Simulated handwritten-style signature of a fictional physician · For teaching use

Complete Blood Count Report

Report details

Field	Value
Report number	ASC-L01
Specimen	EDTA-anticoagulated whole blood
Collection time	09-01 09:45
Report time	09-01 10:15

Complete blood count results

Test	Result	Flag	Unit	Reference interval / Interpretation threshold
White blood cell count	7.8		10^9/L	3.5-9.5
Neutrophil percentage	78	↑	%	40-75
Neutrophil count	6.08		10^9/L	1.8-6.3
Lymphocyte percentage	13	↓	%	20-50
Lymphocyte count	1.01	↓	10^9/L	1.1-3.2
Monocyte percentage	7		%	3-10
Eosinophil percentage	1.5		%	0.4-8
Basophil percentage	0.5		%	0-1
Red blood cell count	3.65	↓	10^12/L	4.3-5.8
Hemoglobin	118	↓	g/L	130-175
Hematocrit	0.35	↓	L/L	0.4-0.5
Mean corpuscular volume	95.9		fL	82-100
Mean corpuscular hemoglobin	32.3		pg	27-34
Mean corpuscular hemoglobin concentration	337		g/L	316-354
Platelet count	68	↓	10^9/L	125-350

Testing method: Routine methods of the simulated laboratory.

Reference intervals are set by this simulated laboratory; ↑ indicates above the upper reference limit, and ↓ indicates below the lower reference limit. For tests without a uniform reference interval, only results are reported; arrows are not assigned arbitrarily.

Test performed by: Lin Yan Reviewer: Su He (role identifiers; simulated signatures for teaching)

SIM-ASC-20260901 | Simulated teaching case · Not a real medical document



SOURCE SECTION 8

Simulated Teaching Hospital

Medical record copy format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record no.: SIM-ASC-20260901

Laboratory Department · Recorded by: Lin Yan Reviewer: Su He

Simulated handwritten-style signature of a fictional physician · For teaching use

Liver and Kidney Function and Electrolyte Laboratory Report

Report details

Field	Value
Report no.	ASC-L02
Specimen	Serum
Collection time	09-01 09:45
Report time	09-01 10:40

Laboratory results

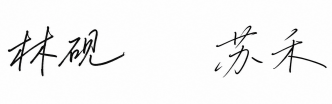
Test	Result	Flag	Unit	Reference interval / Interpretation threshold
ALT	38		U/L	9-50
AST	56	↑	U/L	15-40
Total bilirubin	48	↑	μmol/L	0-21
Direct bilirubin	26	↑	μmol/L	0-7
Albumin	25	↓	g/L	40-55
Total protein	61	↓	g/L	65-85
ALP	112		U/L	45-125
GGT	78	↑	U/L	10-60
Urea	10.2	↑	mmol/L	3.1-8.0
Creatinine	118	↑	μmol/L	57-97
Sodium	129	↓	mmol/L	135-145
Potassium	4.6		mmol/L	3.5-5.3
Chloride	97	↓	mmol/L	99-110
Total carbon dioxide	22		mmol/L	22-29
Glucose	5.8		mmol/L	3.9-6.1
LDH	180		U/L	120-250

Testing method: Simulated routine laboratory methods

The serum sample for this albumin measurement was collected before albumin infusion and paired with the ascitic fluid albumin measurement from the same day. Previous creatinine: 78 μmol/L (08-20); the rising creatinine trend must be assessed within the relevant time window to determine whether acute kidney injury (AKI) is present.

Test performed by: Lin Yan Reviewer: Su He (role identifiers; simulated signatures for teaching)

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Simulated Teaching Hospital

Medical record copy format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record no.: SIM-ASC-20260901

Laboratory Department · Recorded by: Lin Yan Reviewer: Su He

Simulated handwritten-style signature of a fictional physician · For teaching use

Coagulation and Inflammatory Marker Laboratory Report

Report information

Field	Value
Report no.	ASC-L03
Specimen	Citrate-anticoagulated plasma / serum / venous whole blood
Collection time	09-01 09:45
Report time	09-01 10:45

Coagulation and inflammatory marker results

Test	Result	Flag	Unit	Reference interval / interpretation threshold
Prothrombin time	17.8	↑	s	11-14.5
INR	1.55	↑		0.8-1.2
APTT	38	↑	s	25-35
Fibrinogen	1.8	↓	g/L	2-4
CRP	42	↑	mg/L	0-8
Procalcitonin	0.32	↑	ng/mL	0-0.05
Venous lactate	1.8		mmol/L	0.5-2.2

Testing method: Simulated routine laboratory methods

Inflammatory markers cannot replace ascitic fluid cell counts. INR reflects coagulation test results and is not used alone to predict bleeding risk from low-risk puncture procedures in patients with cirrhosis.

Test performed by: Lin Yan Reviewer: Su He (role identifiers; simulated signatures for teaching)

SIM-ASC-20260901 | Simulated teaching case · Not a real medical document



Simulated Teaching Hospital

Medical record copy format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) | Male | 56 years old | Department: Gastroenterology | Medical record number: SIM-ASC-20260901

Laboratory Department · Recorded by: Lin Yan · Reviewer: Su He

Simulated handwritten-style signature of a fictional physician · For teaching use

Urinalysis and Etiologic Screening Report

Report details

Field	Value
Report number	ASC-L04
Specimen	Clean-catch midstream urine / Serum
Collection time	09-01 10:00 / 09:45
Report time	09-01 11:20 / 09-02 09:00

Laboratory results

Test	Result	Flag	Unit	Reference interval / Interpretation threshold
Urine specific gravity	1.02			1.005-1.03
Urine protein	Negative			Negative
Urine occult blood	Negative			Negative
Urine red blood cells	1		cells/HPF	0-3
Urine white blood cells	2		cells/HPF	0-5
Granular casts	None seen			None seen
Urine protein/creatinine ratio	0.09		g/g	0-0.15
HBsAg	Positive			Negative
HBeAg	Negative			Negative
Anti-HBe	Positive			Interpret according to infection status
Anti-HBc	Positive			Interpret according to infection status
HBV DNA	<20		IU/mL	Below the lower limit of quantification of 20
Anti-HCV	Negative			Negative
HIV antigen/antibody	Negative			Negative
AFP	4.2		ng/mL	0-7

Test methods: Routine simulated laboratory methods

Routine urinalysis results were reported the same day; HBV DNA and serology results were reported the following day. Qualitative positive results are indicated directly in words, without numerical up/down arrows. A normal AFP level cannot exclude liver cancer.

Test performed by: Lin Yan | Reviewer: Su He (role identifiers; simulated signatures for teaching)

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SOURCE SECTION 11

Simulated Teaching Hospital

Medical record reproduction format / v1.1 / 2026-09-15 / Pending clinical expert review

Patient and administrative information

Field	Value	Field	Value
Name	Wang Jianhua (fictional)	Sex	Male
Age	56 years	Department	Gastroenterology
Medical record number	SIM-ASC-20260901	Department issuing report	Ultrasound
Recorded by	Xu Cheng	Reviewer	Zhou Lan

Simulated handwritten-style signature of a fictional physician · For teaching use

Ultrasound Examination Report

Report details

Field	Value
Report number	ASC-US01
Examination date and time	2026-09-01 10:35
Report time	11:00

Examination

Liver, gallbladder, spleen, portal vein, and localization of ascites; both kidneys and urinary bladder.

Findings (synthetic case setting)

The liver capsule is somewhat irregular, with coarse, heterogeneous parenchymal echogenicity. No definite focal mass is seen. The main portal vein measures approximately 14 mm in internal diameter; color Doppler shows hepatopetal flow, with no definite filling defect. The spleen measures approximately 145 mm in length and 48 mm in thickness. Multiple anechoic fluid collections are present throughout the abdomen and pelvis, including the perihepatic and perisplenic regions and both iliac fossae. A safe puncture window can be selected in the left lower abdomen. No significant bile duct dilatation is seen. Both kidneys are of reasonably normal size and shape, with no dilatation of the collecting systems. No significant urinary retention is evident in the bladder.

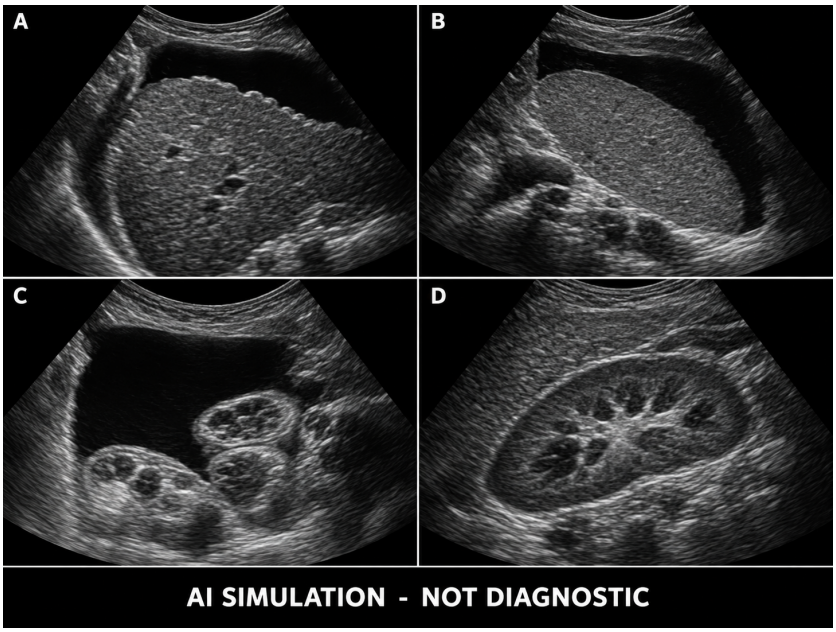
Ultrasound Impression

Sonographic changes of cirrhosis; splenomegaly; large-volume ascites. No definite evidence of portal vein thrombosis, intrahepatic mass, or urinary tract obstruction is seen on this examination.

A: Perihepatic region; B: Perisplenic region; C: Lower abdominal fluid collection; D: Right kidney. AI-generated schematic images, with no actual image acquisition or measurements; not for independent diagnosis.

The images do not include color Doppler. Conclusions regarding portal venous flow and related findings are textual case settings only and cannot be verified from these images. Examination/review: Xu Cheng / Zhou Lan (simulated signatures for teaching).

SIM-ASC-20260901 | Simulated teaching case · Not a real medical document





Simulated Teaching Hospital

Medical Record Copy Format / v1.1 / 2026-09-15 / Pending Clinical Expert Review

Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical Record No.: SIM-ASC-20260901

Electrocardiography Laboratory · Recorded by: Lu Ning Reviewer: Gu Chuan

Simulated handwritten-style signature of a fictional physician · For teaching use

Electrocardiogram Report

Report No.: ASC-ECG01 Examination: 2026-09-01 10:10 Reported: 10:20

Electrocardiogram Parameters

Parameter	Result
Heart rate / Rhythm	96 beats/min / Sinus rhythm
PR / QRS	160 ms / 86 ms
QT / QTc (Bazett)	342 ms / 433 ms
Electrical axis / ST-T	Approximately +45°; no acute ischemic ST-T changes (case specification)

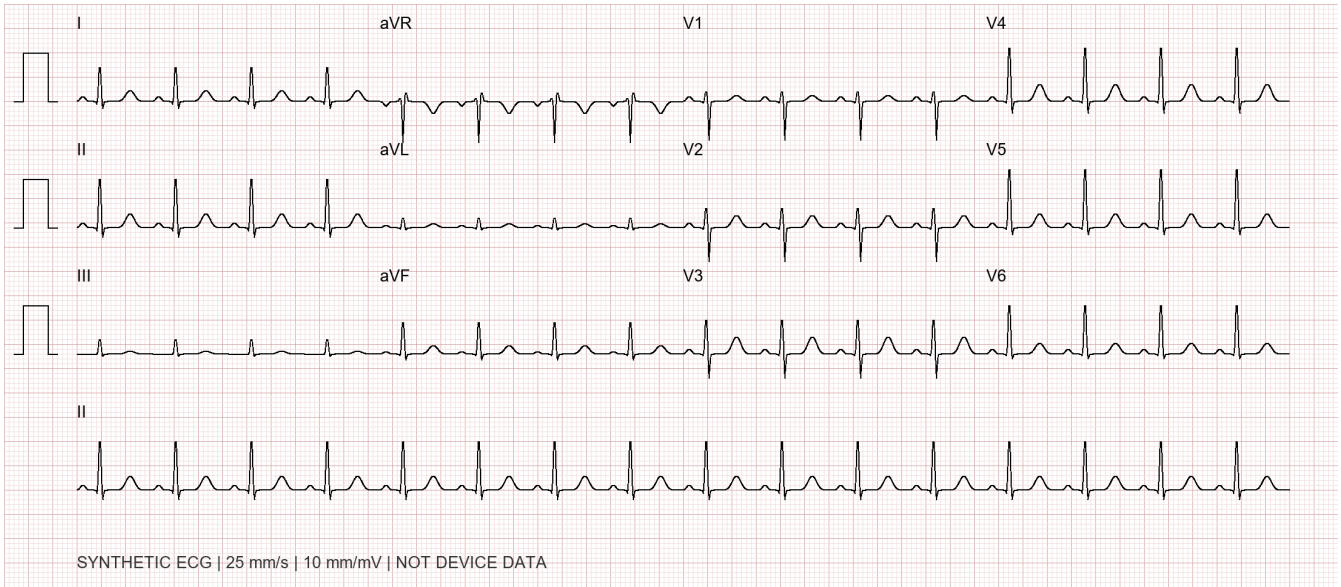
Algorithmically drawn 12-lead schematic, 25 mm/s, 10 mm/mV; not an acquired signal. Intervals and electrical axis are defined by the text specifications; not to be used as a measurement exercise.

Conclusion: Sinus rhythm, with no obvious acute abnormalities. This case has no acute chest pain or signs of heart failure; cardiac biomarkers and echocardiography were not routinely added merely to complete the documentation.

Reporting physician: Lu Ning (simulated signature for teaching)

SIM-ASC-20260901 | Simulated Teaching Case · Not a Real Medical Document





Simulation Teaching Hospital

Medical record reproduction format / v1.1 / 2026-09-15 / Pending clinical expert review

Patient and record details

Field	Value	Field	Value
Name	Wang Jianhua (fictional)	Sex	Male
Age	56 years	Department	Gastroenterology
Medical record number	SIM-ASC-20260901	Recorded by	Zhao Heng, Gastroenterology
Reviewer	Shen Yan		

Simulated handwritten-style signatures of fictional physicians · For teaching use

Diagnostic Paracentesis Procedure Note

Procedure timing

Procedure date	Start time	End time
2026-09-01	11:00	11:15

Indications and Preprocedure Assessment

Hospitalized for worsening ascites associated with cirrhosis, with mild abdominal pain and renal dysfunction; assessment of the nature of the ascites and evaluation for infection were required. The patient was alert, with BP 98/62 mmHg and no evidence of active bleeding; platelet count was 68×10^9/L and INR 1.55. Given the safe access window identified by ultrasound and the low-risk nature of the procedure, prophylactic plasma or platelet transfusion was not given solely on the basis of these values.

Informed Consent Record

The purpose, alternative investigations, and risks, including bleeding, infection, bowel injury, and fluid leakage, were explained to the patient and his wife. The patient understood and consented. This is a simulated record of the consent process, with no real signatures or legal validity.

Procedure Details

After emptying his bladder, the patient was positioned supine with a slight leftward tilt. Ultrasound was used to select an area with sufficient free fluid in the left lower abdomen, avoiding abdominal wall vessels, bowel, and scars. Following routine skin antisepsis and draping, local anesthesia was administered layer by layer using 5 mL of 1% lidocaine (50 mg). The peritoneal cavity was entered using sterile technique, and 40 mL of pale yellow, slightly cloudy ascitic fluid was aspirated.

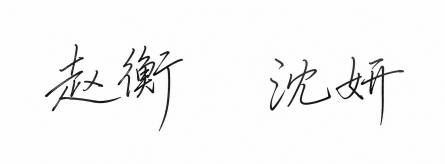
At the bedside, 10 mL of ascitic fluid was inoculated into each of the aerobic and anaerobic blood culture bottles; the remainder was sent separately for cell count and differential and biochemical testing. Ascitic fluid was sampled before ceftriaxone and albumin infusions. After needle withdrawal, local pressure was applied and a sterile dressing placed. Specimen labels were checked, and the specimens were sent immediately for testing.

Postprocedure Status and Orders

The patient had no significant discomfort; BP was 100/64 mmHg and pulse was 94 beats/min. The puncture site was dry, without bleeding or fluid leakage. Monitor abdominal pain, abdominal girth, pulse, blood pressure, and the puncture site. This procedure involved diagnostic sampling only and was not large-volume paracentesis; there was no indication for albumin replacement based on the volume removed. Albumin administered that day was separately intended to manage the risk of SBP-related kidney injury.

Operator: Zhao Heng; Supervisor: Shen Yan; Nurse: Jiang Yue (simulated signatures for teaching)

SIM-ASC-20260901 | Simulated teaching case · Not a real medical document



SOURCE SECTION 14

Simulated Teaching Hospital

Medical record photocopy format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record number: SIM-ASC-20260901

Laboratory Department · Recorded by: Lin Yan Reviewer: Su He

Simulated handwritten-style signature of a fictional physician · For teaching use

Ascitic Fluid Routine Analysis and Biochemistry Report

Report details

Field	Value
Report number	ASC-A01
Specimen	Ascitic fluid
Collection time	09-01 11:05
Report time	09-01 11:45 (cell analysis) / 12:00 (biochemistry)

Ascitic fluid test results

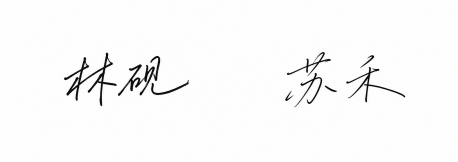
Test	Result	Flag	Unit	Reference interval / Interpretation threshold
Color / Clarity	Pale yellow / Slightly turbid			Descriptive item
Total nucleated cell count	1000		cells/μL	No universally established normal reference interval
Neutrophil percentage	64		%	Interpret in conjunction with the absolute count
Absolute neutrophil count	640	↑	cells/μL	<250; ≥250 is the diagnostic threshold for SBP
Red blood cells	400		cells/μL	Interpret in the context of the paracentesis
Ascitic fluid albumin	7		g/L	Interpret in conjunction with concurrent serum albumin
Ascitic fluid total protein	10		g/L	Interpret in conjunction with the etiology; <15 indicates low-protein ascites
Glucose	5.5		mmol/L	Interpret in conjunction with blood glucose and clinical findings
LDH	72		U/L	Interpret in conjunction with the serum LDH upper limit of 250

Test methods: Cell counting/smear differential and routine biochemistry; all results are simulated.

Nucleated cells 1000 × neutrophils 64% = 640 cells/μL. Serum albumin 25 g/L, ascitic fluid albumin 7 g/L, SAAG = 18 g/L. The PMN arrow is relative to the clinical interpretation threshold, not an ascitic fluid reference interval for healthy individuals. Low protein, glucose that is not decreased, and LDH that is not elevated cannot, on their own, exclude secondary peritonitis.

Test performed by: Lin Yan Reviewer: Su He (role identifiers; simulated signatures for teaching)

SIM-ASC-20260901 | Simulated teaching case · Not a real medical document



Simulated Teaching Hospital

Medical record copy format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) · Male · 56 years old · Department: Gastroenterology · Medical record number: SIM-ASC-20260901

Gastroenterology · Recorded by: Zhao Heng · Reviewer: Shen Yan

Simulated handwritten-style signatures of fictional physicians · For teaching use

Daily Progress Note and Senior Physician Ward Round Record

2026-09-01 12:15

Test Results and Diagnostic Revision

Ascitic fluid PMN count 640 cells/μL, SAAG 18 g/L; ascitic fluid total protein 10 g/L, serum Cr 118 μmol/L, Na 129 mmol/L, and ALB 25 g/L. Portal hypertension-related ascites with spontaneous bacterial peritonitis (SBP) is provisionally established. There are no localized signs of peritoneal irritation, and ascitic fluid biochemistry does not currently support secondary peritonitis; however, the response to treatment must be monitored.

Assessment of Renal Function

Cr has increased from a baseline of 78 μmol/L on 08-20 to the current 118 μmol/L, a 51% increase, but the baseline measurement was more than 7 days ago. Record “suspected AKI” for now; the 7-day criterion cannot be applied directly based solely on this old value. The patient has had oliguria over the past 2 days; infection, inadequate intake, and self-increased diuretic dosing may all be contributing. Repeat testing the next day and short-term trends will be used for confirmation.

Treatment Implementation

Ceftriaxone 2 g by intravenous infusion every 24 hours was started at 11:50. Onset was in the community, with no recent antibiotic use or hospitalization and no known history of antimicrobial resistance. This simulated scenario assumes that the hospital's empirical regimen for community-onset SBP permits this drug; reassess once culture results are available. At 12:10, 20% human albumin was started, calculated using an estimated dry weight of 68 kg × 1.5 g/kg = 102 g (510 mL), to be infused slowly in divided portions while monitoring respiration, SpO2, blood pressure, and pulmonary signs.

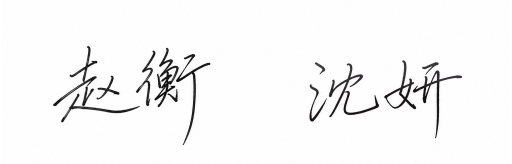
Temporarily withhold diuretics and large-volume paracentesis. Although abdominal distension is marked, there is currently no evidence of respiratory limitation or abdominal compartment syndrome; prioritize infection control and assessment of perfusion. There is no shock, so vasoconstrictors are not being used at present. Restrict dietary sodium to approximately 2 g/day (equivalent to approximately 5 g/day of table salt), while ensuring adequate energy and protein intake. With Na 129 mmol/L and no neurological symptoms, routine strict fluid restriction is not indicated.

Severity

The admission Child-Pugh score comprises 2 points for bilirubin, 3 for albumin, 1 for INR, 3 for large-volume ascites, and 1 for absence of encephalopathy, totaling 10 points, class C. The ascites score is based on clinical judgment. Infection and renal dysfunction increase short-term risk and require close observation.

2026-09-01 18:00: No dyspnea or crackles during the infusion; BP 102/64 mmHg. Abdominal distension persists, with no signs of peritoneal irritation. The family was informed of the key points for overnight observation and the possibility of changes in the patient's condition.

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Simulated Teaching Hospital

Medical record copy format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record number: SIM-ASC-20260901

Gastroenterology · Recorded by: Zhao Heng Reviewer: Shen Yan

Simulated handwritten signature of a fictional physician · For teaching use

Hospital Day 2 Renal Function and Electrolyte Report

Report details

Field	Value
Report number	ASC-L05
Specimen	Serum
Collection time	09-02 06:00
Report time	09-02 07:30

Laboratory results

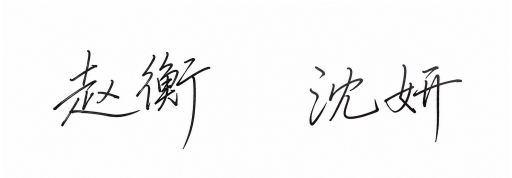
Test	Result	Flag	Unit	Reference interval / Interpretation threshold
Creatinine	150	↑	μmol/L	57-97
Urea	11.8	↑	mmol/L	3.1-8
Sodium	128	↓	mmol/L	135-145
Potassium	4.8		mmol/L	3.5-5.3
Chloride	96	↓	mmol/L	99-110
Total carbon dioxide	21	↓	mmol/L	22-29
Albumin	30	↓	g/L	40-55
Total bilirubin	46	↑	μmol/L	0-21
CRP	38	↑	mg/L	0-8

Test method: Routine simulated laboratory methods

Creatinine rose from 118 μmol/L on the previous day to 150 μmol/L, an increase of 32 μmol/L over approximately 20 hours, meeting the 48-hour absolute increase criterion for AKI. This is 1.92 times the relatively reliable historical baseline of 78 μmol/L; the highest stage currently documented is stage 1.

Test performed by: Lin Yan Reviewer: Su He (role identifiers; simulated signatures for teaching)

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Simulated Teaching Hospital

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Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record number: SIM-ASC-20260901

Gastroenterology · Recorded by: Zhao Heng Reviewer: Shen Yan

Simulated handwritten-style signature of a fictional physician · For teaching use

Hospital Day 2 Progress Note and Nephrology Consultation

2026-09-02 09:00

Changes in Clinical Condition

T 37.1°C, BP 104/66 mmHg, pulse 90/min; 24-hour urine output 900 mL, weight 76.1 kg. The abdomen remains distended, with mild tenderness improved from previously and no rebound tenderness. No melena, altered mental status, hypoxemia, or pulmonary crackles. Cr 150 μmol/L, Na 128 mmol/L.

AKI Diagnosis

Cr increased from 118 the day before yesterday to 150 currently, an increase of 32 μmol/L within 48 hours. Using the known stable value of 78 μmol/L on 08-20 as the baseline for staging, the peak is 1.92 times baseline, indicating stage 1 AKI. A daily urine output of 900 mL alone cannot establish that oliguria criteria were met for 6 consecutive hours; the diagnosis in this case is based on creatinine.

Consultation Opinion (10:30)

The AKI is considered to have been triggered by infection, with reduced effective circulating volume; recent poor oral intake and a self-directed increase in diuretic dosage may have contributed. Urine sediment shows no significant active abnormalities, proteinuria is minimal, and ultrasound shows no obstruction. There is currently insufficient evidence to support glomerulonephritis or overt tubular necrosis, but these findings alone cannot completely exclude them. HRS-AKI remains a differential diagnosis requiring follow-up and cannot yet be diagnosed solely on the basis of “cirrhosis + AKI.”

Management and Reassessment

Continue withholding diuretics, continue antimicrobial treatment, and closely record intake and output. Determine whether further fluid administration is needed based on peripheral perfusion, blood pressure, pulmonary assessment, and bedside volume assessment. As day 1 albumin administration for SBP has already been completed and there is no shock, do not automatically add a fixed 48-hour course of volume expansion. Do not initiate terlipressin in the absence of a definite diagnosis of HRS-AKI. Recheck renal function and electrolytes within 24 hours, or earlier if the condition worsens.

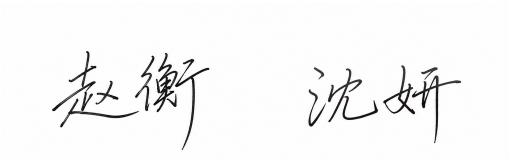
At 16:00, preliminary ascitic fluid culture findings suggest gram-negative bacilli; antimicrobial susceptibility results are not yet available. Blood cultures have shown no growth so far. Continue the current regimen and monitor.

Antiviral Medication Review

At the peak creatinine of 150 μmol/L, CrCl was roughly estimated at approximately 47 mL/min using estimated dry weight. Entecavir was temporarily adjusted to 1 mg every 48 hours: the dose was taken on 09-01; omit one dose on 09-02 and reassess renal function the following day.

Consultant: Chen Jin; recorded by: Zhao Heng (simulated signature for teaching)

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Simulated Teaching Hospital

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Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record no.: SIM-ASC-20260901

Laboratory Department · Recorded by: Lin Yan Reviewer: Su He

Simulated handwritten-style signature of a fictional physician · For teaching use

Ascitic Fluid Bacterial Culture and Antimicrobial Susceptibility Report

Report details

Field	Value
Report no.	ASC-M01
Specimen	Ascitic fluid inoculated at bedside into aerobic/anaerobic culture bottles
Collection time	09-01 11:05
Report time	09-03 09:30 (final report)

Culture and antimicrobial susceptibility results

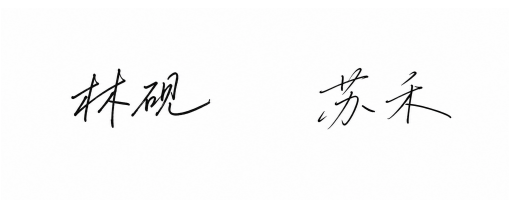
Test	Result	Flag	Unit	Reference interval / Interpretive threshold
Culture	Escherichia coli	Positive; single organism		
Ceftriaxone	Susceptible (S)			Simulated susceptibility category
Piperacillin/tazobactam	Susceptible (S)			Simulated susceptibility category
Meropenem	Susceptible (S)			Simulated susceptibility category
Ciprofloxacin	Susceptible (S)			Simulated susceptibility category
Trimethoprim/sulfamethoxazole	Susceptible (S)			Simulated susceptibility category
Ampicillin	Resistant (R)			Simulated susceptibility category
ESBL screen	Negative			Negative

Test method: Simulated routine laboratory method

Preliminary report at 09-02 16:00: Gram-negative bacilli; final identification and susceptibility report on 09-03. S/R are preset categories for this fictional case; no instrument MIC values or laboratory breakpoint version have been fabricated. The specimen was collected before the first antimicrobial dose.

Test performed by: Lin Yan Reviewer: Su He (role identifiers; simulated signatures for teaching)

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Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record number: SIM-ASC-20260901

Laboratory Department · Recorded by: Lin Yan Reviewer: Su He

Simulated handwritten signature of a fictional physician · For teaching use

Hospital Day 3 Follow-up Laboratory Report

Report number: ASC-L06 Specimens: Serum / EDTA whole blood / Ascitic fluid

Collection time: 09-03 06:00 (blood) / 11:50 (ascitic fluid) Report time: 09-03 07:30 / 12:30

Follow-up laboratory results

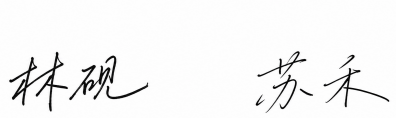
Test	Result	Flag	Unit	Reference interval / Interpretation threshold
White blood cell count	5.6		10^9/L	3.5-9.5
Hemoglobin	116	↓	g/L	130-175
Platelet count	70	↓	10^9/L	125-350
Creatinine	112	↑	μmol/L	57-97
Urea	9.5	↑	mmol/L	3.1-8
Sodium	130	↓	mmol/L	135-145
Potassium	4.5		mmol/L	3.5-5.3
CRP	20	↑	mg/L	0-8
Ascitic fluid total nucleated cell count	300		cells/μL	No uniform reference interval
Ascitic fluid neutrophil percentage	40		%	Interpret in conjunction with the absolute count
Ascitic fluid absolute PMN count	120		cells/μL	<250; ≥250 is the diagnostic threshold for SBP

Test methods: Simulated routine laboratory methods

Repeat paracentesis was performed approximately 48 hours after the first dose of ceftriaxone; the PMN count decreased from 640 to 120 cells/μL, a reduction of 81.25%. The blood specimen was collected before the albumin infusion on Day 3.

Test performed by: Lin Yan Reviewer: Su He (role identifiers; simulated signatures for teaching)

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Simulated Teaching Hospital

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Name: Wang Jianhua (fictional) · Male · 56 years old · Department: Gastroenterology · Medical record no.: SIM-ASC-20260901

Gastroenterology · Recorded by: Zhao Heng · Review: Shen Yan

Simulated handwritten-style signatures of fictional physicians · For teaching use

Hospital Day 3 Progress Note and Repeat Paracentesis Record

2026-09-03 09:00 / 11:45–12:00 / 14:00

Ward Round Findings

No fever. BP 106/66 mmHg, pulse 84 beats/min, urine output 1300 mL/24 h, weight 75.8 kg. Abdominal pain has resolved, but abdominal distension remains relatively pronounced. No signs of peritoneal irritation, encephalopathy, or dyspnea. Cr decreased from 150 to 112 μmol/L; Na 130 mmol/L.

Repeat Paracentesis

Repeat assessment of treatment response was planned because renal function had previously deteriorated in this case. The procedure was explained again, and simulated informed consent was obtained. A safe window in the left lower abdomen was selected using ultrasound. Under aseptic conditions, local anesthesia was administered with 3 mL of 1% lidocaine, and 20 mL of ascitic fluid was aspirated for differential cell count and culture. The needle was withdrawn, pressure applied, and a dressing placed. Blood pressure remained stable before and after the procedure, with no bleeding, fluid leakage, or significant abdominal pain. Operator: Zhao Heng; supervisor: Shen Yan.

Interpretation of Results

PMN count was 120 cells/μL, an 81.25% decrease from 640 and below 250. Culture grew ceftriaxone-sensitive Escherichia coli. Symptoms and renal function improved concurrently, supporting the effectiveness of the current antimicrobial regimen. There is no evidence requiring escalation to a carbapenem. There are no new clinical clues suggesting secondary peritonitis; contrast-enhanced CT will not be performed at present. If the response is inadequate, reassess the etiology and source of infection.

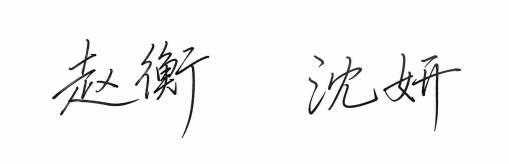
Treatment

Continue ceftriaxone 2 g once daily, with an anticipated total course of 7 days (09-01 through 09-07). Administer 68 g (340 mL) of 20% albumin on day 3 according to the spontaneous bacterial peritonitis (SBP) regimen, in divided infusions with monitoring. Continue withholding diuretics and recheck renal function the next day. If perfusion is stable, therapeutic paracentesis may be considered for symptom relief.

Short-term improvement in renal function does not justify declaring that all kidney diseases have been excluded. However, the current recovery trajectory does not support listing HRS-AKI as a confirmed discharge diagnosis.

At a creatinine level of 112 μmol/L, roughly estimated CrCl is approximately 63 mL/min. Resume entecavir 1 mg orally once daily on an empty stomach.

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Simulated Teaching Hospital

Medical record copy format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record number: SIM-ASC-20260901

Clinical Laboratory · Recorded by: Lin Yan Reviewed by: Su He

Simulated handwritten signature of a fictional physician · For teaching use

Pre-paracentesis Safety Assessment Laboratory Report

Report details

Field	Value
Report number	ASC-L07
Specimen	Serum / Citrated plasma / EDTA whole blood
Collection time	09-04 06:00
Report time	09-04 07:30

Laboratory results

Test	Result	Flag	Unit	Reference interval / Interpretation threshold
Creatinine	96		μmol/L	57-97
Urea	7.6		mmol/L	3.1-8
Sodium	132	↓	mmol/L	135-145
Potassium	4.3		mmol/L	3.5-5.3
Albumin	32	↓	g/L	40-55
Total bilirubin	38	↑	μmol/L	0-21
INR	1.45	↑		0.8-1.2
Platelets	72	↓	10^9/L	125-350

Testing method: Simulated routine laboratory methods

Given the absence of bleeding on physical examination, hemodynamic stability, and a safe ultrasound window, the decision was made to perform therapeutic paracentesis. The albumin level is affected by prior infusion and is not considered standalone evidence of short-term recovery of hepatic synthetic function.

Test performed by: Lin Yan Reviewer: Su He (role identifiers; simulated signatures for teaching)

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Simulated Teaching Hospital

Medical record reproduction format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) | Male | Age: 56 years | Department: Gastroenterology | Medical record no.: SIM-ASC-20260901

Gastroenterology · Recorded by: Zhao Heng · Reviewer: Shen Yan

Simulated handwritten-style signatures of fictional physicians · For teaching use

Therapeutic Paracentesis Record

2026-09-04 | Start: 10:00 | End: 11:40

Pre-procedure Diagnosis and Indications

Cirrhosis with large-volume ascites; good response to treatment for spontaneous bacterial peritonitis (SBP), with recovery of renal function. Persistent marked abdominal distension, early satiety, and a tense abdominal wall. BP 108/68 mmHg, pulse 82 beats/min, SpO2 98%. Currently does not meet the definition of refractory ascites.

Informed Consent and Preparation

The purpose of fluid removal, albumin replacement, and risks including bleeding, infection, hypotension, circulatory dysfunction, and fluid leakage were explained; the patient consented. Identity and the ultrasound window were verified, and the bladder was emptied. No prophylactic plasma or platelet transfusion was given.

Procedure

The patient was placed in a semi-recumbent position. Ultrasound was used to select a safe fluid pocket in the left lower abdomen, avoiding abdominal wall vessels. Following routine sterile skin preparation and draping, local anesthesia was administered with 5 mL of 1% lidocaine, and a single-use paracentesis catheter was inserted. A total of 6000 mL of clear, pale-yellow ascitic fluid was slowly drained with observation at intervals. Vital signs and symptoms were assessed every 15 minutes during the procedure. No catheter was left in place for continuous drainage. Upon completion, the catheter was removed, pressure was applied, and a sterile dressing was placed.

Albumin and Safety Record

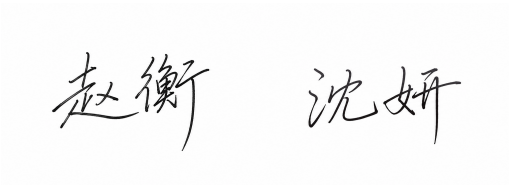
Based on 6 L of fluid removed × 8 g/L, an additional 48 g (240 mL) of 20% human albumin was administered by slow intravenous infusion after completion of the procedure, with monitoring. This was for the indication of ascitic fluid removal and was documented separately from the SBP regimens on 09-01 and 09-03; it was not duplicate same-day counting.

Post-procedure

Final BP 106/66 mmHg, pulse 80 beats/min, SpO2 98%. Abdominal distension was markedly relieved. Abdominal circumference decreased from 105 cm to 96 cm, and body weight decreased from 75.6 kg to 69.6 kg. There was no bleeding or fluid leakage at the site, no worsening abdominal pain, and no syncope. Continue observation for 2 hours and recheck renal function and electrolytes the following morning.

Performing physician: Zhao Heng; supervising physician: Shen Yan; nurse: Jiang Yue (all simulated roles). No surgery was performed in this case; this is an invasive procedure record.

SIM-ASC-20260901 | Simulated teaching case · Not a real medical document



Simulated Teaching Hospital

Medical record reproduction format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record number: SIM-ASC-20260901

Gastroenterology · Recorded by: Zhao Heng Reviewer: Shen Yan

Simulated handwritten-style signature of a fictional physician · For teaching use

Progress Notes for Hospital Days 4–6

09-04 17:00 · Post-paracentesis ward round

The patient can eat and lie flat relatively comfortably. BP 108/68 mmHg. The puncture site is dry, with no fever or abdominal pain. No signs of pulmonary edema. The recorded 6000 mL was fluid removed during the procedure; the 6 kg decrease in body weight that day must not be interpreted as a diuretic effect. Continue monitoring; do not add further diuretics for now.

09-05 09:00 · Resumption of low-dose diuretics

T 36.7°C, BP 110/68 mmHg, weight 69.4 kg, urine output 1400 mL/24 h. Cr 88 μmol/L, Na 133 mmol/L, K 4.2 mmol/L. A small amount of residual ascites and mild ankle edema remain. Resume spironolactone 50 mg once daily and furosemide 20 mg once daily, both orally. Start at low doses because of the recent AKI; there is no current need to aim for maximum doses. Weigh daily under the same conditions and monitor blood pressure, urine output, creatinine, and electrolytes.

09-06 09:00 · Treatment response and nutrition

T 36.6°C, BP 110/70 mmHg, weight 69.0 kg, urine output 1800 mL/24 h. Mild ankle edema; no recurrence of abdominal pain; normal bowel movements. Weight decreased by 0.4 kg from the previous day, with good tolerance. The final blood culture report at 5 days detected no bacteria. The repeat ascitic fluid culture from 09-03 has not yet been finalized; do not incorrectly document it as already negative.

Nutrition and Activity Plan

Based on an estimated dry weight of 68 kg, plan approximately 2040–2380 kcal/day and approximately 82–102 g/day of protein, with small, frequent meals and a bedtime snack. Both edema and signs of muscle loss are present; body weight that includes ascitic fluid cannot be used directly to judge nutritional adequacy. Routine protein restriction is unnecessary. Encourage walking and light resistance activity as tolerated.

Diet, body weight, and medication doses should be jointly checked by medical and nursing staff. A low-sodium diet does not mean a completely salt-free diet. In the absence of severe hyponatremia and neurological symptoms, routine strict fluid restriction is not indicated.

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SOURCE SECTION 24

Simulated Teaching Hospital

Medical record copy format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record number: SIM-ASC-20260901

Laboratory Department · Recorded by: Lin Yan Reviewer: Su He

Simulated handwritten-style signature of a fictional physician · For teaching use

Post-treatment Renal Function and Pre-discharge Laboratory Report

Report details

Field	Details
Report number	ASC-L08
Specimens	Serum / EDTA whole blood / Citrate plasma
Collection times	09-05 / 09-07 / 09-09 at 06:00
Report times	07:30 on the corresponding dates

Laboratory results

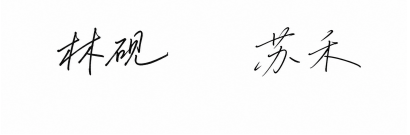
Test (collection date)	Result	Flag	Unit	Reference interval / Interpretation threshold
09-05 Creatinine	88		μmol/L	57-97
09-05 Sodium	133	↓	mmol/L	135-145
09-05 Potassium	4.2		mmol/L	3.5-5.3
09-07 Creatinine	84		μmol/L	57-97
09-07 Sodium	134	↓	mmol/L	135-145
09-07 Potassium	4.1		mmol/L	3.5-5.3
09-07 CRP	5		mg/L	0-8
09-09 Creatinine	82		μmol/L	57-97
09-09 Urea	5.6		mmol/L	3.1-8
09-09 Sodium	134	↓	mmol/L	135-145
09-09 Potassium	4.2		mmol/L	3.5-5.3
09-09 Albumin	32	↓	g/L	40-55
09-09 Total bilirubin	30	↑	μmol/L	0-21
09-09 INR	1.35	↑		0.8-1.2
09-09 White blood cell count	4.2		10^9/L	3.5-9.5
09-09 Hemoglobin	119	↓	g/L	130-175
09-09 Platelet count	75	↓	10^9/L	125-350

Testing methods: Routine simulated laboratory methods

This is a simulated LIS consolidated printout covering multiple time points. Each test retains its collection date; these results are not from a single blood draw. Na 134 remains below this laboratory's lower limit. The discharge diagnosis should state “improved,” not “completely normal.”

Test performed by: Lin Yan Reviewer: Su He (role identifiers; simulated signatures for teaching)

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Simulation Teaching Hospital

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Name: Wang Jianhua (fictional) Sex: Male Age: 56 years Department: Gastroenterology Medical record number: SIM-ASC-20260901

Clinical Laboratory · Recorded by: Lin Yan Review: Su He

Simulated handwritten-style signature of a fictional physician · For teaching use

Final Microbiology Culture Reports

Microbiology culture reports

Report number / Time	Specimen / Result
ASC-M02 / 09-06 10:00	Two sets of peripheral blood cultures collected on 09-01 at 09:50; no bacteria detected after 5 days of incubation.
ASC-M03 / 09-08 12:30	Repeat ascitic fluid culture collected on 09-03 at 11:50; no bacteria detected after 5 days of incubation.

All microbiology results are synthetic case data; negative cultures alone cannot establish that infection has been ruled out in the absence of cell counts and clinical assessment.

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Patient and administrative information

Field	Value	Field	Value
Name	Wang Jianhua (fictional)	Sex	Male
Age	56 years	Department	Gastroenterology
Medical record number	SIM-ASC-20260901	Department issuing report	Ultrasound
Recorded by	Xu Cheng	Reviewer	Zhou Lan

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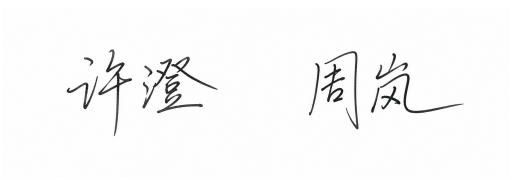
Pre-discharge Ultrasound Examination Report

ASC-US02 · Follow-up Ultrasound Report, 09-08 09:30

The morphology of the liver and spleen is similar to that on the previous examination. A small amount of free fluid is present in the abdomen and pelvis, markedly decreased from before. No definite new focal hepatic lesion or thrombus in the main portal vein is seen. The collecting systems of both kidneys are not dilated.

Purpose of examination: To assess residual ascites and renal status. Only a written report is provided for this follow-up examination; the schematic image of large-volume ascites at admission is not reused as a purported follow-up image.

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Simulated Teaching Hospital

Medical Record Copy Format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) | Male | 56 years old | Department: Gastroenterology | Medical record number: SIM-ASC-20260901

Gastroenterology · Recorded by: Zhao Heng · Reviewer: Shen Yan

Simulated handwritten-style signatures of fictional physicians · For teaching use

Progress Notes for Hospital Days 7–8

09-07 09:00 Progress Note

Weight 68.6 kg, BP 112/70 mmHg, urine output 1700 mL/24 h; abdominal pain and edema have resolved. Cr 84, Na 134, K 4.1, CRP 5. Ceftriaxone was discontinued after completion of day 7 of treatment. No prior history of SBP; SBP was confirmed during this episode and has resolved. Assess initiation of secondary prophylaxis.

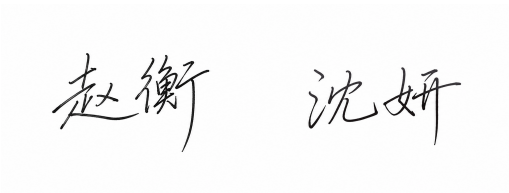
09-08 09:00 Progress Note

Weight 68.3 kg, BP 110/68 mmHg, urine output 1500 mL/24 h; oral intake and sleep have improved. After considering antimicrobial susceptibility results, renal function, drug interactions, and the risk of adverse reactions, ciprofloxacin 500 mg orally once daily was started for secondary prophylaxis of SBP. The patient was advised to contact a physician promptly for tendon pain, peripheral nerve symptoms, severe diarrhea, or similar symptoms. This regimen must be reassessed in light of resistance patterns and tolerability.

Follow-up Arrangements

Repeat renal function and electrolyte testing is scheduled for 09-12; a liver disease/liver transplant assessment clinic appointment is scheduled for 09-16 to discuss long-term strategies following decompensation. Neither TIPS nor liver transplantation is documented as treatment completed during this admission.

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Simulated Teaching Hospital

Medical record copy format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record number: SIM-ASC-20260901

Gastroenterology · Recorded by: Zhao Heng Reviewer: Shen Yan

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Medication Orders and Administration Summary

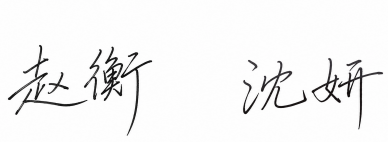
Medication orders and administration

Medication	Dose / Route / Frequency	Timing	Purpose and verification
Ceftriaxone	2 g / Intravenous infusion / Every 24 hours	09-01 11:50 through 09-07; 7 administrations in total	SBP treatment; continued after culture confirmed susceptibility; stopped after completion on 09-07
20% human albumin	102 g = 510 mL / Intravenous infusion / One course administered in divided portions	09-01	SBP day 1; estimated dry weight 68 × 1.5; monitor infusion tolerance
20% human albumin	68 g = 340 mL / Intravenous infusion / One course administered in divided portions	09-03	SBP day 3; 68 × 1; not intended to normalize serum albumin
20% human albumin	48 g = 240 mL / Intravenous infusion	After ascitic fluid drainage on 09-04	6 L of ascitic fluid drained × 8 g/L; documented on a separate day from the SBP regimen
Spironolactone	50 mg / Oral / Once daily	Held 09-01 through 09-04; resumed 09-05	Monitor serum potassium, creatinine, and blood pressure; restarted at a low dose in this case
Furosemide	20 mg / Oral / Once daily	Held 09-01 through 09-04; resumed 09-05	Combined with spironolactone; do not increase the dose independently
Entecavir	1 mg / Oral / Once daily on an empty stomach	Taken 09-01; one dose held 09-02; resumed 09-03	Temporarily changed to every 48 hours at the peak of AKI; 1 mg/day after recovery; continue after discharge
Ciprofloxacin	500 mg / Oral / Once daily	Starting 09-08; reassessment 09-16	Secondary prophylaxis for SBP; not an extension of the intravenous course for acute infection
1% lidocaine	5/3/5 mL / Local infiltration / Once each	During procedures on 09-01 / 09-03 / 09-04	50/30/50 mg, respectively; no other local anesthetics on the same days

This is a synthetic administration summary of the medications actually used for this simulated patient, not an expanded HIS medication catalog. Dilution, infusion rate, and formulation specifications for each intravenous medication must follow local prescribing information and pharmacy review; do not fabricate batch numbers.

For entecavir, when creatinine peaked at 150 μmol/L, a rough Cockcroft–Gault estimate using a 56-year-old male with an estimated dry weight of 68 kg was approximately 47 mL/min; however, estimates during AKI are unstable. Following assessment by the clinical pharmacist that day, the regimen was temporarily changed to 1 mg every 48 hours (one dose held on 09-02). On 09-03, after creatinine decreased to 112, 1 mg/day was resumed. See the verification notes on the next page for details.

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Medical record copy format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record number: SIM-ASC-20260901

Gastroenterology · Recorded by: Zhao Heng Reviewer: Shen Yan

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Medication Review and Nonpharmacological Orders

Entecavir Dose Adjustment Details

Previous long-term dose: 1 mg/day. During this episode, 1 mg had already been taken on 09-01. On 09-02, when creatinine peaked at 150, CrCl was roughly estimated at approximately 47 mL/min. According to the prescribing information, the dose for decompensated disease at a CrCl of 30–49 mL/min may be 1 mg every 48 hours; therefore, no dose was taken on 09-02. On 09-03, creatinine was 112 and CrCl was roughly estimated at approximately 63 mL/min; 1 mg/day was resumed. CrCl estimates during AKI have limitations and require ongoing reassessment; no dialysis was performed in this case. An empty stomach means at least 2 hours before or after a meal.

Antibacterial Medication Review

Ceftriaxone is the empirical regimen specified in this case for community-onset infection, supported by susceptibility testing. The susceptibility results in this case must not be generalized to actual local resistance rates. Ciprofloxacin is used for secondary prophylaxis of SBP; resistance, QT-related risks, tendon/neurological adverse effects, and the risk of Clostridioides difficile must be assessed. Separate administration from preparations containing polyvalent cations such as calcium, iron, and magnesium. Recheck dosing if renal function worsens.

Diuretic Safety

Check blood pressure, weight, urine output, creatinine, and Na/K at initiation and after every dose adjustment. Daily weight loss should generally not exceed 0.5 kg in the absence of peripheral edema, or 1 kg when edema is present; a slower rate of loss is the target in this case. The 6 kg decrease due to fluid drainage on 09-04 is recorded separately. If dizziness, oliguria, rising creatinine, marked hyponatremia, or hyperkalemia/hypokalemia occurs, promptly withhold or adjust treatment and investigate the cause.

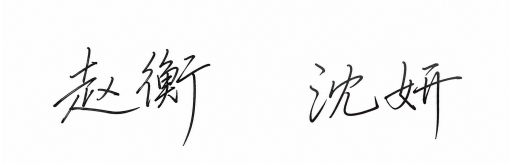
Nonpharmacological Orders

Gastroenterology nursing care; monitor vital signs every 4 hours early in the admission, with more frequent monitoring during albumin infusion and procedures. Weigh daily on the same scale after morning urination, measure abdominal circumference at a consistent anatomical site, and record intake and output. Restrict sodium to approximately 2 g/day while ensuring adequate nutrition; individualize water intake according to serum sodium and volume status. Encourage activity as tolerated and implement fall prevention.

Treatments Not Used in This Case

No hepatic encephalopathy: lactulose/rifaximin is not routinely given. No gastrointestinal bleeding: no hemostatic agents or empirical PPI are given. No confirmed diagnosis of HRS-AKI: terlipressin is not used. No clear evidence of refractory ascites: TIPS is not planned. The prophylactic strategy for previously documented small varices awaits specialist reassessment after hemodynamic stabilization.

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Simulated Teaching Hospital

Medical Record Copy Format / v1.1 / 2026-09-15 / Pending Clinical Expert Review

Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical Record No.: SIM-ASC-20260901

Gastroenterology Nursing Team · Recorded by: Jiang Yue Reviewer: Song Qing

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Nursing Records and Clinical Trends

Daily Nursing Records and Clinical Trends

Date	Weight (kg)	Abdominal Circumference (cm)	BP (mmHg)	Urine Output (mL)	Key Events
09-01	76.0	106	98/62	900	Diagnostic paracentesis: 40 mL; albumin: 102 g
09-02	76.1	106	104/66	900	AKI confirmed; diuretics remained withheld
09-03	75.8	105	106/66	1300	Repeat paracentesis: 20 mL; albumin: 68 g
09-04	75.6→69.6	105→96	108/68	1400	Ascitic fluid drained: 6000 mL; albumin: 48 g
09-05	69.4	96	110/68	1400	Low-dose dual-diuretic therapy resumed
09-06	69.0	95	110/70	1800	Well tolerated
09-07	68.6	95	112/70	1700	Completed 7 days of intravenous antimicrobial therapy
09-08	68.3	94	110/68	1500	Secondary prophylaxis for SBP initiated
09-09	68.1	94	112/70	1500	Discharged

Urine output represents the most recent 24 hours before rounds on the specified day; the 09-01 value was estimated by the patient, and subsequent values were recorded on the ward. Weight and abdominal circumference on 09-04 show pre- and post-procedure values. No indwelling urinary catheter was used; urine output was recorded using a dedicated measuring cup.

Nursing Observations

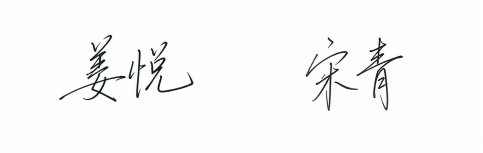
The patient remained alert throughout hospitalization, with no hematemesis, melena, or falls. All puncture sites remained dry, with no persistent fluid leakage or obvious hematoma. There was no acute dyspnea, hypoxemia, or crackles during albumin infusions.

Health Education and Teach-Back

The patient was able to repeat: “I must not increase my diuretic doses on my own; I should weigh myself daily under the same conditions; I should seek medical attention promptly if I have a marked decrease in urine output, fever and abdominal pain, black stools, or changes in consciousness.” His wife participated in organizing medications and planning meals.

Nursing recorder: Jiang Yue; Checked by: Song Qing (simulated signatures for teaching)

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Medical record copy format / v1.1 / 2026-09-15 / Pending clinical expert review

Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record no.: SIM-ASC-20260901

Gastroenterology · Recorded by: Zhao Heng Reviewer: Shen Yan

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Discharge Summary

Discharge date and time: 2026-09-09 10:00

Condition on Admission

Admitted for recurrent abdominal distension for 6 months, worsening for 10 days, with decreased urine output for 2 days. On admission, weight was 76 kg, with massive ascites, bilateral lower-extremity edema, and BP 98/62 mmHg. Ascitic fluid PMN count was 640 cells/μL and SAAG was 18 g/L; subsequent culture grew Escherichia coli.

Hospital Course

Abdominal paracentesis was performed promptly, and ceftriaxone was administered for 7 days along with albumin according to the SBP regimen. On day 2, Cr rose from 118 to 150 μmol/L, confirming stage 1 AKI. Diuretics were withheld, perfusion and the causes of kidney injury were assessed, and the entecavir dosing interval was temporarily adjusted. On day 3, the ascitic fluid PMN count decreased to 120 and Cr was 112. On day 4, after Cr reached 96, therapeutic paracentesis removed 6 L of ascitic fluid, with 48 g of albumin replacement. Low-dose spironolactone/furosemide was resumed from day 5, and renal function remained stable. Secondary prophylaxis for SBP began on day 8.

Discharge Diagnoses

1. Decompensated hepatitis B cirrhosis with ascites. 2. Spontaneous bacterial peritonitis (Escherichia coli, controlled). 3. Stage 1 acute kidney injury (resolved; associated with infection and reduced effective circulating volume). 4. Hyponatremia (improved). 5. Portal hypertension and splenomegaly with thrombocytopenia. 6. Hypoalbuminemia.

Condition at Discharge

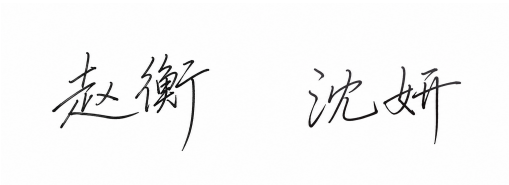
Temperature 36.6°C, pulse 78 beats/min, BP 112/70 mmHg; alert, with improved oral intake, no abdominal pain, a small amount of residual ascites, and no significant peripheral edema. Weight 68.1 kg; Cr 82 μmol/L, Na 134 and K 4.2 mmol/L, ALB 32 g/L, TBil 30 μmol/L, and INR 1.35. Creatinine was only 4 μmol/L above the previous baseline, consistent with a recovery trajectory.

Severity and Outcome

At discharge, the Child-Pugh score comprised bilirubin 1, albumin 2, INR 1, a small amount of ascites controlled with diuretics 2, and no encephalopathy 1, totaling 7 points, class B. Recent albumin infusion affects interpretation of the score. Clinical improvement does not mean that cirrhosis has been cured. There is no evidence to establish a diagnosis of HRS-AKI or refractory ascites.

See the next page for discharge instructions and follow-up. Recording physician: Zhao Heng; supervising physician: Shen Yan (simulated signatures for teaching).

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Name: Wang Jianhua (fictional) Male 56 years old Department: Gastroenterology Medical record number: SIM-ASC-20260901

Gastroenterology · Recorded by: Zhao Heng Reviewer: Shen Yan

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Discharge Medications and Follow-up Plan

Discharge medications

Medication	Directions	Purpose / Review
Entecavir	1 mg orally once daily on an empty stomach	Continue antiviral therapy; do not discontinue on your own. Monitor renal function and HBV DNA.
Spironolactone	50 mg orally once daily	Control ascites; monitor creatinine, serum potassium, serum sodium, and blood pressure.
Furosemide	20 mg orally once daily	Control ascites; do not increase the dose on your own.
Ciprofloxacin	500 mg orally once daily; initially prescribed through the follow-up visit on 09-16	Secondary prophylaxis for spontaneous bacterial peritonitis (SBP); prescription renewal to be determined at follow-up. This must not be interpreted as requiring only 7 days of prophylaxis.

Upcoming Reassessment

09-12 (3 days after discharge): Check blood pressure, weight, renal function, and Na/K, and assess diuretic tolerability; seek care earlier if abnormalities occur. 09-16: Attend the liver disease specialist clinic and liver transplant evaluation clinic; repeat complete blood count, liver function tests, and coagulation tests, and assess ascites, nutrition, and secondary prophylaxis medications.

Long-term Management

Maintain a low-sodium diet with adequate energy and protein intake, small frequent meals, and a bedtime snack; abstain from alcohol. Weigh yourself daily and keep a record. Avoid using NSAIDs or Chinese herbal remedies of unknown origin on your own. Attend regular antiviral follow-up visits and monitor HBV DNA according to the specialist's plan. Liver cancer surveillance generally consists of ultrasound plus AFP every 6 months; the next surveillance is planned for 2027-03, with earlier further investigation if abnormalities occur. Previously documented varices and tolerability of nonselective beta-blockers (NSBBs) will be reassessed at an upcoming specialist visit.

When to Seek Prompt Medical Care

Seek immediate medical care for fever or new abdominal pain, markedly reduced urine output, persistent dizziness/fainting, vomiting blood or black stools, drowsiness or abnormal behavior, or difficulty breathing. Contact the follow-up physician as soon as possible if abdominal girth and weight continue to increase or appetite declines again. Tendon pain, numbness, or severe diarrhea requires prompt review of ciprofloxacin therapy.

Confirmation of Patient Understanding

The patient and his wife were able to repeat the medication names, the instruction not to increase diuretic doses on their own, and the reassessment dates. It was explained that the condition still requires long-term management and that liver transplant evaluation does not mean a decision to proceed with transplantation has already been made.

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